

Solano County Health & Social Services Department

Mental Health Services
Public Health Services
Substance Abuse Services
Older & Disabled Adult Services



Eligibility Services
Employment Services
Children's Services
Administrative Services

Patrick O. Duterte, Director

EMERGENCY MEDICAL SERVICES AGENCY

Michael A. Frenn
EMS Agency Administrator

275 Beck Ave., MS 5-240
Fairfield, CA 94533
(707) 784-8155 FAX (707) 421-6682
www.solanocounty.com

Richard C. Lotsch, D. O.
EMS Agency Medical Director

POLICY MEMORANDUM 2208

DATE: 02/16/2006

REVIEWED/APPROVED BY:


RICHARD C. LOTSCH, D.O., EMS AGENCY MEDICAL DIRECTOR


MICHAEL A. FRENN, EMS AGENCY ADMINISTRATOR

SUBJECT: CONTINUOUS QUALITY IMPROVEMENT (CQI) COMMITTEE CASE PRESENTATION

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, §1797.204; 1797.220.

I. PURPOSE/POLICY:

To establish the minimum standards for CQI Case Presentations at CQI Committee Meetings.

II. INTENT

A case presentation is a regular and core activity of the CQI Committee. It is intended that these presentations will be both informative to the Committee as a whole and reflective of the sophistication of the presenting organization's CQI efforts.

III. CONTENT

- A. A case presentation should include the following:
 - 1. Reason selected
 - a. Randomly audited;
 - b. Unusual
 - c. Problematic
 - d. Educational Value
 - 2. A concise description of the issue(s);
 - 3. Prehospital Care Report (PCR)
 - 4. Base Hospital Tape (if relevant)
 - 5. Base Hospital Documentation Form
 - 6. Base Physician report of hospital diagnosis, treatment and outcome
 - 7. Specific factors that caused the case to be reviewed.
 - 8. County policies and protocols involved in the case;
 - 9. Follow-up procedures and outcomes;
 - 10. Internal changes, if any, in practices or procedures made by the presenting agency.
 - 11. Recommend changes, if any, in EMS system policies and procedures;
 - 12. Reporting Agency's Medical Director's written analysis of the issues, solutions and outcomes.

IV. PROCEDURE:

- A. A schedule of assigning each ALS Provider and the EMS Agency a month for a presentation will occur in November of each year for the coming year.
- B. All relevant data concerning the case presentation will be submitted to the EMS Agency two (2) weeks prior to the CQI meeting date for review.
- C. Case presentations will be closed to all but designated primary or alternate CQI Committee Members and EMS Agency Personnel.
 - 1. The member agency must submit to the EMS Agency, and keep current, the names of their designated primary and alternate members.
 - 2. All CQI case presentation material is guarded, i.e., distributed and collected at meetings, confidential, and will conform to the provisions of the Health Information Protection Act (HIPPA);
 - 3. With the exception of generic, educational material, the Committee Chair will collect all written materials provided to committee members at the end of the presentation; only educational material may be kept by members.

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Aaron E. Bair, MD, MSc
EMS Agency Medical Director

Ted Selby
Emergency Services Bureau Chief

POLICY MEMORANDUM 2220

Implementation Date: 6/20/13

Release Date: 6/20/13

REVIEWED/APPROVED BY:

A handwritten signature in blue ink, appearing to read "Bair MD".

AARON E. BAIR, MD, MSc, EMS AGENCY MEDICAL DIRECTOR

A handwritten signature in blue ink, appearing to read "T. Selby".

TED SELBY, EMS AGENCY ADMINISTRATOR

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.103, 1797.199, 1798.161, 1799.51

SUBJECT: EMERGENCY MEDICAL SERVICES (EMS) DATA COLLECTION & REPORTING

PURPOSE/POLICY:

The purpose of the policy is to establish the requirements for EMS data collection and reporting functions for all EMS Providers and Hospitals serving Solano County in order to provide the EMS Agency with the data and comparative analysis tools necessary to assess and improve the quality of the EMS System.

Data Requirements for the Solano County EMS System

I. Field Data Collection and Reporting

Each Ambulance Service and/or first responder ALS Service Provider will provide EMS system data collection and reporting functions according to the following specifications set forth by the Solano County EMS Agency Medical Director and/or

A. Monthly Submission of Pre-hospital Care Report (PCR) Data

By the 15th of each month, each Provider will submit to the EMS Agency a copy of the previous month's PCRs.

B. ALS and CCT/SCT Ambulance Service and/or Service Provider

1. Field Data Elements Required by the EMS Agency

Each first responder ALS and/or CCT/SCT service provider shall submit CEMSIS compliant data elements to the EMS Agency by the 15th of each month for the previous month's data.

See the EMS Data System Standards (Per the Emergency Services Authority) for the data elements and their definitions:

<http://www.emsa.ca.gov/pubs/pdf/EMSA164.pdf>

II. Trauma Data Collection and Reporting

Each Trauma Center designated by Solano County and/or utilized as part of the Solano County trauma system will provide EMS system data collection and reporting functions according to the following specifications set forth by the Solano County EMS Agency Medical Director and/or designee.

A. Trauma Dataset Patient Inclusion Criteria (per CEMSIS requirements)

A trauma patient is defined as a patient sustaining a traumatic injury and meeting the following criteria:

1. at least one of the following injury diagnostic codes defined in the International Classification of Diseases, Ninth Revision, Clinical Modification (ICD-9-CM):

800 – 959.9

2. **AND:**

Physically evaluated by trauma or burn surgeon in the ED or resuscitation area

OR

Death in the Emergency Department

OR

Transfer for trauma services (note: may include inter-facility and intra-facility)

3. **Exclusion:** *Isolated burn without penetrating or blunt mechanism of injury*

B. Trauma Data Elements Required by the EMS Agency

Each Trauma Center designated by Solano County shall submit CEMSIS compliant trauma data elements to the EMS Agency by the 15th of each month for the previous month's data. See the CEMSIS Trauma Data Dictionary for the data elements and their definitions:

<http://www.emsa.ca.gov/systems/Trauma/files/TraumaDictionary11152011.pdf>

III. ST-segment Elevation Myocardial Infarction (STEMI) Data Collection and Reporting

Each STEMI Receiving Center (SRC) designated by Solano County and/or utilized as part of the Solano County STEMI system will provide EMS system data collection and reporting functions according to the following specifications set forth by the Solano County EMS Agency Medical Director and/or designee.

A. STEMI Data Elements Required by the EMS Agency

Each SRC shall submit STEMI case level data to the EMS Agency by the 15th of each month for the previous month's data. See Attachment 1 for the data elements and reporting format.

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Attachment 1: STEMI Data Collection for STEMI Receiving Centers

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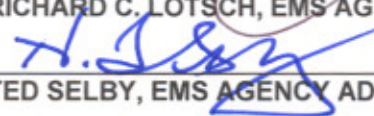
Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 2305

DATE: February 17, 2011

REVIEWED/APPROVED BY:


RICHARD C. LOTSCH, EMS AGENCY MEDICAL DIRECTOR


TED SELBY, EMS AGENCY ADMINISTRATOR

SUBJECT: FIELD ADVISORY REPORT (FAR)

AUTHORITY: CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1797.220.

I. PURPOSE/POLICY:

The purpose of this policy is to formalize a mechanism for identifying and reviewing problems or potential problems, as well as positive issues, for the express purpose of continually improving the delivery of prehospital care in Solano County.

II. INCIDENTS REQUIRING A FIELD ADVISORY REPORT (FAR)

A. The following circumstances require submission of a FAR to the EMS Agency:

1. A situation that adversely, or potentially adversely, affects a patient;
2. A conflict exists with a County Policy/Protocol and/or state law, or a patient's condition warrants deviation from protocol requiring Base Physician's order to do so;
3. A significant threat or barrier to the delivery of high quality patient care;
4. A threat to the safety of patients or providers of prehospital care;
5. Noteworthy performance benefiting the patient, the Emergency Medical Services (EMS) system, or the community.
6. A request by the EMS Agency.

III. REPORTING PARTY**A. Any person or agency may initiate a FAR.**

A report shall be made whenever a person or agency decides, in good faith, that an inquiry, investigation, corrective action, or positive recognition may be warranted. It is not the responsibility of the reporting party to determine blame, accountability, or whether the occurrence is intentional, avoidable, preventable, or inappropriate.

B. Base Hospitals.

Base Hospitals shall submit FARs on incidents involving their areas of oversight, (e.g., field care, personnel, or incidents occurring at the Base Hospital), including:

1. Incidents of noteworthy benefit to the patient, EMS system, or community
2. Deviation from policy/protocol
3. Deviation from destination guidelines
4. Controlled substance irregularities
5. Unprofessional conduct

C. Ground and Air Providers.

Ground and air providers shall be responsible for completing and submitting FARs associated with incidents involving their personnel, including:

1. Items of noteworthy benefit to the patient, EMS system, or community
2. Deviation from policy/protocol
3. Dispatch issues
4. Deviation from destination guidelines
5. Controlled substance irregularities
6. Unprofessional conduct
7. Equipment failure/malfunction
8. Deviation from authorized supplies/equipment
9. Complications/unexpected clinical events
10. Documentation issues

D. The EMS Agency.

The EMS Agency may generate and investigate, or request the generation of a FAR, on any issue, event, or occurrence that it deems necessary or beneficial to individuals, patients, agencies, facilities, or the EMS system as a whole.

IV. REPORTING PROCESS

The reporting party shall complete the approved *EMS Agency Field Advisory Report Form* and submit the FAR to the EMS Agency within 72 hours of the time the incident occurred.

A. FAR Form

1. Reporting party shall only use the approved *EMS Agency Field Advisory Report Form* when submitting a FAR to the EMS Agency.
2. All reports shall be as complete as possible, including incident dates, times, and locations, as well as complete reporting party contact information.
3. Incomplete or incorrect forms will be returned to the reporting party for completion or correction prior to Agency action.

V. INVESTIGATING AGENCY

The EMS Agency is the primary investigating agency of a FAR, and is responsible for the collection of all data appropriate to the investigation.

VI. EMS AGENCY PROCESS

A. Incident Opened

1. The FAR will be logged in the *Field Advisory Log Book* with the following information:
 - a. Log number
 - b. Log date
 - c. Basic information regarding the incident
 - d. Date reported
2. Review Process
 - a. The EMS Agency will review the FAR to determine the level of review or investigation required.

B. Investigation

1. The EMS Agency is the primary investigative agency for all FARs.
 - a. The EMS staff member will conduct the investigation.
 - b. Individuals, facilities, agencies, and organizations may be required to provide information and documentation pertinent to the investigation to the EMS Agency.
 1. Individuals, facilities, agencies, and organizations must meet requests by the Agency for information within 48 hours.
 2. The Agency may require individuals, facilities, agencies, and organizations to conduct secondary investigations and report their findings of fact with recommendations to the EMS Agency within a reasonable period of time.

VII. OUTCOMES, RECOMMENDATIONS, DIRECTIVES, AND ACTIONS BY THE EMS AGENCY

- A. The EMS Agency is responsible for investigative findings and actions, and will determine whether investigative findings and actions taken were appropriate and adequately completed. The case will then be marked “closed” in the Field Advisory Log Book.
- B. If formal counseling is required by the employer or EMS Agency, the individual(s) involved will be requested by the counselor to appear at a time and date agreed upon. Once requested, counseling is obligatory. Due process and representation will be respected by the counseling agency.
- C. Written counseling documentation will be sent to the EMS Agency. If the EMS Agency determines that the counseling did not adequately address the problem, the Agency may also counsel the individual involved.
- D. Action on EMT-Basic certifications is the sole responsibility of the EMS Agency. Action on paramedic licenses is the responsibility of the State EMS Authority.
- E. In cases where the Exclusive Operating Area (EOA) Agreement is breached by non-exclusive operators, the EMS Agency shall investigate and levy fines and penalties as appropriate.

VIII. CONFIDENTIALITY

- A. Employees do not need permission or clearance from their employers to submit a FAR to the EMS Agency.
- B. All FARs are confidential and will not be a part of a patient's medical record.
- C. All FARs, investigation materials, and procedures will conform to the Health Insurance Portability and Accountability Act (HIPAA).
- D. Individuals, facilities, agencies and organizations concerned in a FAR will take reasonable measures to maintain the confidentiality of all the parties involved.

IX. QUARTERLY REPORT

- A. A report shall be prepared on a quarterly basis.
- B. The report shall include the following:
 - 1. Total number of FARs received during the current quarter.
 - 2. Total number of incidents currently in the review process.
 - 3. Total number of incidents reviewed and closed during the current quarter.



Health Services Department

Public Health Division

Donald R. Rowe, Director

Thomas L. Charron, M.D., M.P.H.
Health Officer
Assistant Director

EMERGENCY MEDICAL SERVICES

POLICY MEMORANDUM #2310

EFFECTIVE DATE: 06/01/92

APPROVED BY:

THOMAS L. CHARRON, M.D., M.P.H., SOLANO COUNTY HEALTH OFFICER, EMS MEDICAL DIRECTOR

REVIEWED BY:

STEVEN J. KAMENSKI, M.D., F.A.C.E.P., ASSISTANT EMS MEDICAL DIRECTOR

AUTHORITY:

CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, 1798.200;
CALIFORNIA ADMINISTRATIVE CODE, TITLE 22, DIVISION 9,
CHAPTER 6.

SUBJECT: CERTIFICATE REVIEW PROCESS FOR PREHOSPITAL PERSONNEL

PURPOSE/POLICY:

To establish a mechanism for, and the explanation of, the certification review process. The certification review process shall be consistent with Chapter 6 of the California Administrative Code.

I. DEFINITIONS

- A. Affected Personnel - are defined as any individual who is authorized/certified in Solano County to perform prehospital EMS services. To include, when applicable:
1. EMD (Emergency Medical Dispatch)
 2. EMT-I (Emergency Medical Technician I)
 3. EMT-II (Emergency Medical Technician II)
 4. MICN (Mobile Intensive Care Nurse)
- B. "Investigative Review Panel" or "IRP" - means an impartial advisory body, the members of which are knowledgeable in the provision of prehospital emergency medical care and local EMS system policies and procedures, which may be convened to review allegations against the holder of an EMS prehospital emergency medical care certificate, assist in establishing the

care certificate, assist in establishing the facts of the matter, and provide its findings to the EMS Medical Director of a local EMS Agency.

- C. “Special Investigative Review Panel” – is an IRP that may be convened by the EMS Agency if requested by an affected EMS certificate holder. The EMS certificate holder must have had one or more EMS certificates “immediately suspended” by the EMS Medical Director or Assistant EMS Medical Director. The affected individual must, within fifteen (15) calendar days of the date of immediate suspension, request in writing a special IRP. A special IRP must adhere to the requirements established in Section 100220 of the California Code of Regulations. The use of a “Special Investigative Review Panel” is limited to cases when the holder of an EMS certificate is “immediately suspended.” Other negative certification actions allowed under 100215 shall follow the IRP process described under Section 100222.

II. SANCTIONS

The EMS Medical Director or Assistant EMS Medical Director may institute the following formal proceedings against prehospital care medical personnel in Solano County when there exists an imminent threat to public health and safety:

- A. Placement of a certification authorization holder on probation.
- B. Suspension of certification/authorization.
- C. Revocation of certification/authorization.
- D. Denial of certification/authorization.
- E. Denial of renewal of certification/authorization.

III. DISCIPLINARY ACTION

Disciplinary action will be instituted based upon evidence of a threat to the public health and safety including any of the following:

- A. Fraud in the procurement of any certification/authorization.
- B. Gross negligence;
- C. Repeat of negligent acts;
- D. Incompetence;
- E. The commission of any fraudulent, dishonest, or corrupt act which is related to the qualifications, functions and duties of prehospital personnel.
- F. Conviction of any crime which is related to the qualifications, functions and duties of prehospital personnel. The record of conviction or certified copy thereof shall be conclusive evidence of such conviction.
- G. Violating or attempting to violate, any provision of the Health & Safety Code or of regulations promulgated by the authority pertaining to prehospital personnel;
- H. Violating or attempting to violate any federal or State statute or regulation which regulates narcotics, dangerous drugs or controlled substances as defined in California Health & Safety Code 11054 through 11058.

- I. Addiction to or abuse of alcoholic beverages, narcotics, dangerous drugs or controlled substances as defined in Section III, H, above.
- J. Functioning outside the scope of the held certificate/authorization or independent of medical control in the field care system operating at the local level, except as authorized by any other license or certification/authorization.
- K. Failure to pass a competency based examination or to meet any other requirements for certification/authorization or continuation of certification/authorization.

IV. ALLEGATION EVALUATION

Any allegation received from a credible source, including discovery through medical audit, of any action by the holder of, or applicant for, a prehospital EMS certificate/authorization, which, if found to be true would be evidence of a threat to the public health and safety will be evaluated by the EMS Medical Director. Any action taken by a local EMS Agency of another jurisdiction or State, may be accepted by the EMS Medical Director without further review.

V. IMMEDIATE SUSPENSION

The EMS Medical Director and/or the Assistant EMS Medical Director may immediately suspend a prehospital emergency medical care certificate/authorization at any point in the investigative or appeal process when substantial evidence exists which, in the expert opinion of the EMS Medical Director or the Assistant EMS Medical Director, indicates that a clear, present, immediate and continuing threat to the public health & safety would exist if the certificate/authorization were not suspended.

- A. If the subject's certificate/authorization is immediately suspended prior to the initiation or completion of a review of all allegations by an Investigative Review Panel (IRP) as described below, the subject may, within fifteen (15) calendar days of the date of suspension, request that a special IRP be convened to review the facts which necessitate an immediate suspension. Immediate suspension is not limited to a single action. The cumulative history of inappropriate actions may be considered a cause of immediate suspension.
- B. Upon receipt of a request for a special IRP, the EMS Medical Director may convene a special IRP, unless the circumstances under Section V, D apply, to review the facts which necessitate an immediate suspension of the individual's certificate/authorization prior to EMS Staff completion of the investigation and/or the IRP.
- C. The special IRP review of the facts necessitating the immediate suspension shall be completed and the subject notified of the IRP's recommendations and the EMS Medical Director's decision within twenty-one (21) calendar days of receipt of the request for the special IRP.
- D. The special IRP does not have to be convened if an IRP review of all the facts of the matter can be held and the subject notified of the IRP's recommendations and the EMS Medical Director's decision within twenty-one (21) calendar days of receipt of the request for the special IRP.

VI. INVESTIGATIVE PROCESS

The investigative process by the EMS Office shall include the following:

- A. The EMS Medical Director or his/her designee shall first evaluate the allegation(s) relative to the potential threat to the public health and safety and determine if further action is warranted.
- B. The EMS Medical Director or his/her designee shall review all available pertinent information pertaining to the allegation(s).
- C. The EMS Medical Director or his/her designee shall determine the seriousness of the allegation(s) relative to the potential threat to the public health and safety, determine the possible disciplinary action which could be taken if the allegations are found to be substantiated, and initiate a formal investigation.
- D. The subject of the investigation and the subject's employer(s) shall be notified in writing within fifteen (15) calendar days of the initiation of a formal investigation and shall be allowed to submit information in writing and to meet informally with the EMS Medical Director or his/her designee to review the allegations.
- E. The subject and his/her employer shall be allowed a maximum of fifteen (15) calendar days to respond to the request of the EMS Medical Director or his/her designee for information unless extenuating circumstances preclude response within that time, and the EMS Medical Director feels that an extension of the response time would not jeopardize the public health and safety.
- F. The written notice to the subject and his/her employer shall include:
 - 1. A statement of the allegations against the subject.
 - 2. A statement which explains that the allegations, if found to be true, constitute a threat to the public health and safety and necessitate that the EMS Medical Director take disciplinary action pursuant to Section 1798.200 of the Health & Safety Code.
 - 3. An explanation of the possible actions which may be taken if the allegations are found to be true.
 - 4. A brief explanation of the formal investigation process.
 - 5. A request for a written response to the allegations from the subject.
 - 6. A statement that the subject may submit in writing any information which she/he feels is pertinent to the investigation, including statements from other individuals, etc.
 - 7. A statement that if she/he so chooses, the subject may designate another person, including legal counsel or subject's employer, to represent him/her during the investigation.
 - 8. The date by which the information must be submitted.
 - 9. An explanation of the IRP process if suspension, revocation, denial or denial of renewal of a certificate occurs.

- G. The EMS Medical Director or his/her designee shall designate a person or persons to assure that any and all relevant information pertaining to the allegation(s) and to the performance of the subject in regard to the use of prehospital emergency medical care skills is gathered.
- H. The EMS Medical Director may determine that the infraction or performance deficiency is relative to the potential threat to the public health and safety and may institute disciplinary action without calling a review panel. The subject shall be informed that she/he may request an IRP review if the disciplinary action is suspension, revocation, denial or denial of renewal of a certificate/authorization.

VII. AN IRP SHALL BE CONVENED WHEN:

- A. The EMS Medical Director determines that an IRP is necessary to assist in the investigation of the allegations.
- B. The subject submits a written request to the EMS Medical Director within fifteen (15) calendar days of the notification of disciplinary action taken, when the disciplinary action is: suspension, revocation, denial or denial of renewal of a certificate/authorization.
- C. If a special IRP has been requested and the EMS Medical Director elects to convene and IRP within twenty-one (21) calendar days of receipt of the request for a special IRP.
- D. A special IRP review has occurred, unless the subject specifies in writing that she/he does not want a further IRP review of the facts of the case as required by Section 100222.

VIII. THE IRP:

- A. Shall be held within thirty (30) calendar days of receipt of request by the subject. The EMS Medical Director has the option to convene an IRP without request from the subject. Within five (5) calendar days of the selection of the IRP, the subject and the subject's employer shall be advised in writing of the purpose of the review panel, its composition, and the date and time that it will convene. Any change in the time or date of convening the IRP shall be mutually agreed upon in writing by both the subject and the EMS Medical Director or his/her designee.
- B. Shall consist of three (3) persons knowledgeable in the provision of prehospital emergency care and local EMS system policies and procedures. One member shall be mutually agreed upon by the subject and the EMS Medical Director.
- C. Shall not include anyone who submitted allegation(s) against the subject or who was directly involved in the incident(s).
- D. Shall assess all of the available information on the matter in order to establish the facts of the case. The subject shall be given the opportunity to be present during the presentation of any testimony before the IRP, and may choose to be represented either by legal counsel or another representative of his/her choosing.

- E. Shall make a written report of its findings and its recommendation for disciplinary action to the EMS Medical Director within fifteen (15) calendar days of the close of the IRP Hearing.
- F. Proceedings will be closed to the public and witnesses will be required to wait outside the hearing room except when testifying.
- G. Will ensure that the subject's due process rights are protected during the hearing process. The Chair of the IRP shall control the hearing. Members of the IRP will be able to obtain legal counsel during the hearing process if a question regarding due process is raised (see Attachment A).
 - 1. The EMS Medical Director or his/her designee shall present the case against the subject and explain the rationale for taking disciplinary action.
 - 2. The subject shall be given the opportunity to testify, cross-examine witnesses, or present additional evidence or witnesses at the hearing.
 - 3. The subject may be represented by legal counsel, or another representative of his/her choosing, at the hearing.
 - 4. During the hearing, an relevant evidence, including hearsay evidence, may be admitted if it is the sort of evidence which responsible persons are accustomed to relying on in conduct of serious affairs.
- H. All pertinent documentary evidence gathered in the investigation process shall be provided to the subject and/or his representative for his/her review at least five (5) calendar days before the scheduled hearing.
- I. The subject may request the presence at the hearing of any person from whom evidence or testimony was taken during the initial investigation in order to cross examine that person. If the requested person cannot attend the hearing, a written statement from that person may be given to the IRP panel and the subject. The subject shall have the right to review the written statement and to provide a written rebuttal. Both statements shall be admitted as evidence. The Panel shall review all evidence as presented. At the commencement of the hearing, the Chair shall determine the order of evidence to be presented.

IX. IRP ACTION

The EMS Medical Director will determine what action, if any, should be taken as a result of the findings of the investigative process. Actions which the EMS Medical Director may take include one or more of the following:

- A. No disciplinary action – If the allegation(s) are found to be untrue, unsubstantiated or unrelated to the ability of the subject to perform his/her duties as a prehospital care provider, the EMS Medical Director may choose to take no disciplinary action.
- B. Mandatory Audit – The subject will be informed that his/her conduct in the field will be monitored and actively reviewed by the EMS Medical Director and/or his/her designee for a specified period of time.

- C. Counseling and/or mandatory education – the EMS Medical Director may designate another qualified person, such as the person's employer or medical supervisor, to provide the specified counseling, and/or the EMS Medical Director may mandate further appropriate education. The counseling session(s) to occur within a time limit set by the EMS Medical Director or his/her designee shall include a review of the findings of the investigation, the specific issues of concern, the improvement of the subject, the manner(s) in which such improvement may be achieved, and the evaluation method which will be used to assess the subject's improvement. If further education is required, courses must be pre-approved by the EMS Medical Director.
- D. Mandated preceptorship – The EMS Medical Director or his/her designee may require the accredited/certified personnel to function with a designated preceptor appointed by the EMS Medical Director or his/her designee to assist and monitor performance. The EMS Medical Director or his/her designee and the individual employer shall agree as to how the preceptorship shall be completed.
- E. Written Reprimand – the EMS Medical Director or his/her designee may issue a written reprimand if the facts in the case indicate that the subject has committed a minor infraction which is unlikely to recur, is not representative of the subject's usual behavior, and is not likely to continue to jeopardize the public health and safety. A copy of the reprimand shall be placed in the subject's personnel file at the local EMS Agency and a copy shall be forwarded to his/her employer.
- F. Probation – If the seriousness of the infraction or performance deficiency indicates a need for further scrutiny of the subject's conduct, the EMS Medical director or the Assistant EMS Medical Director may place the subject on probation for a specific period of time not to exceed one year. Probation may be chosen in addition to specific remediation, counseling, education and/or reprimand, at the discretion of the EMS Medical Director or Assistant EMS Medical Director. The subject's performance shall be reviewed as prescribed in the probationary contract.
- G. Suspension – If the seriousness of the infraction of performance deficiency indicates a need to suspend the subject from the practice of prehospital emergency medical care, but the findings indicate that the subject need not be indefinitely prevented from that practice in order to protect public health and safety, the EMS Medical Director or the Assistant EMS Medical Director may suspend the subject's certification/ authorization by the usual process as allowed by the IRP process in this policy.
- H. Revocation, denial or denial of renewal – If the seriousness of the infraction or performance deficiency is of such a serious nature that it is likely that the holder of the certificate/authorization cannot practice without posing a significant or continuing risk to the public health and safety, the EMS Medical Director may revoke, deny, or deny the renewal of a certificate/authorization.

X. NOTIFICATION OF ACTION

The EMS Medical Director or his/her designee shall formally notify the subject and the subject's employer of the disciplinary action taken. The notification shall be in writing and shall include the following:

- A. The specific allegations which resulted in the investigation.

- B. A summary of the findings of the investigation, including the findings and recommendations of the IRP, if one was convened.
- C. The disciplinary actions taken and the effective date of the action, including the duration, if the action includes counseling, further education, probation or suspension.
- D. Which certificate/authorization(s) the action applies to if more than one certificate/authorization is held by the subject.
- E. If no IRP was convened, and the action taken is suspension, revocation, denial or denial of renewal, the subject's right to request that an IRP, or a special IRP if immediate suspension has occurred, review both the allegations against him/her and the disciplinary action proposed.
- F. If the action taken is probation, suspension, revocation, denial or denial of renewal, a statement that the subject must report these actions taken if she/he applies for any certification or authorization from another local agency.
- G. Records of formal disciplinary proceedings and actions will be maintained by EMS for three (3) years following the completion of the proceedings.

Solano County Health & Social Services Department



Gerald Huber, Director

Aaron Bair, MD, MS
EMS Agency Medical Director

EMERGENCY MEDICAL SERVICES AGENCY
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Ted Selby
EMS Agency Administrator

POLICY MEMORANDUM 2315

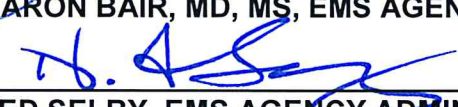
Implementation Date: May 10, 2012

Revised Date: June 15, 2016

Review Date: June 1, 2018

REVIEWED/APPROVED BY:


AARON BAIR, MD, MS, EMS AGENCY MEDICAL DIRECTOR


TED SELBY, EMS AGENCY ADMINISTRATOR

**SUBJECT: SOLANO COUNTY EMERGENCY MEDICAL SERVICES (EMS)
POLICY & PROTOCOL DEVELOPMENT & REVIEW**

AUTHORITY:

CALIFORNIA HEALTH & SAFETY CODE, DIVISION 2.5, § 1797.220

PURPOSE:

Establish procedures for development and update of Solano County Emergency Medical Services (EMS) Policies and Protocols.

I. SOLANO COUNTY EMS POLICIES & PROTOCOLS

Solano County EMS Policies will be organized into following general topic areas:

- Section 1000 – The Organization
- Section 2000 – Quality Assurance
- Section 3000 – EMS Personnel
- Section 4000 – Initial Training & Education Providers
- Section 5000 – EMS Providers
- Section 6000 – Patient Care
- Section 7000 – Facilities
- Section 8000 – Continuous Quality Improvement
- Appendices

Solano County EMS Protocols are developed for the treatment of patients using Advanced Life Support (ALS) and Basic Life Support (BLS) skills.

II. POLICY & PROTOCOL DEVELOPMENT

- A. Solano County EMS Policies and Protocols are developed in accordance with the California Health & Safety Code, Div 2.5; Section 1797.220.
- B. New policies/protocols, and changes to current policies/protocols, may be initiated by either field personnel or Solano County EMS staff. Field personnel must coordinate with their Provider Medical Director prior to request for any changes, updates, or new policies or protocols.
- C. New policies/protocols or updates may come from the EMS Agency, Process Improvement, Physicians' Forum, STEMI, Trauma Advisory Committee or any Solano County EMS provider. These suggestions will be discussed in committee (as appropriate) and EMS Staff may develop a draft policy/protocol for review and additional comment. Specific tasks and responsibilities are included in Attachment 1, Policy & Protocol Development & Review Flowchart.

III. POLICY & PROTOCOL REVIEW

- A. All Solano County EMS Policies and Protocols will be reviewed on a biennial basis from the last date of revision.
 - 1. If no revisions are necessary to the Policy and/or Protocol under review, the review date will be changed to two years from the current review date.
 - 2. If revisions are necessary to the Policy and/or Protocol under review, the procedure in Section II of this policy will be followed to make the revisions.
 - 3. Policies and/or Protocols may be revised at any time if new standards, practices, or regulations are enacted or by suggestion from the committees outlined in Section II(C) of this policy.

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Policy 2315
Solano County EMS Policy & Protocol Development & Review Flowchart
Attachment 1

